

VirtualWorks Pharmacovigilance Internship

Task 1 – ADR Identification Basics

Prepared by: Nagesh Dongare

Case ID: SIM-ADR-001

Educational simulated case

1. Patient Case Scenario

A 50-year-old male patient was treated for a bacterial infection with **ciprofloxacin 500 mg orally twice daily**. On the day treatment was started, he developed **generalized urticaria (hives)**. Ciprofloxacin was stopped and the patient received treatment for the allergic reaction. The patient subsequently recovered. Paracetamol was also being used as a concomitant medication. No relevant previous drug-allergy history was reported.

2. Patient and Medication Information

Field	Information
Patient	50-year-old male
Suspected drug	Ciprofloxacin
Dose / route	500 mg orally, twice daily
Indication	Bacterial infection
Adverse event	Generalized urticaria (hives)
Onset	Same day as starting ciprofloxacin
Concomitant medication	Paracetamol
Action taken	Ciprofloxacin withdrawn; treatment for allergic reaction given
Outcome	Recovered

3. ADR Identification

Identified ADR: Generalized urticaria (hives), suspected to be associated with ciprofloxacin.

Suspected drug: Ciprofloxacin.

Why it is considered an ADR: The reaction appeared after exposure to the suspected medicine, the timing is clinically compatible with a drug-related reaction, and the medicine was withdrawn after the reaction.

4. Timeline Assessment

Time point	Event
Before treatment	Patient had bacterial infection; no reported relevant drug-allergy history.
Treatment started	Ciprofloxacin 500 mg orally twice daily was started.
Same day	Generalized urticaria developed.
After reaction	Ciprofloxacin was stopped and treatment for the reaction was given.
Follow-up	Patient recovered.

5. Assessment of Possible Alternative Causes

An infectious illness itself can sometimes produce skin manifestations, and concomitant medicines should also be considered. However, the close temporal relationship between ciprofloxacin exposure and onset of urticaria makes ciprofloxacin a reasonable suspected drug. The available information is not sufficient to prove that ciprofloxacin was the only possible cause.

6. Final Task 1 Conclusion

ADR identified: Generalized urticaria (hives).

Suspected medicine: Ciprofloxacin 500 mg orally twice daily.

Relationship: Suspected drug-related adverse reaction based on the temporal relationship and clinical presentation.

Case type: Educational simulated case for internship practice.

7. Important Originality Note

This report is written as an original educational simulation rather than a copy of a published case report. For the VirtualWorks submission, present your own analysis and wording. Do not upload another person's YouTube video or copy a published case report word-for-word.

8. References

1. World Health Organization. Pharmacovigilance: ensuring the safe use of medicines.
2. Indian Pharmacopoeia Commission. Pharmacovigilance Programme of India: Guidance Document for Spontaneous Adverse Drug Reaction Reporting.
3. WHO-Uppsala Monitoring Centre. The use of the WHO-UMC system for standardised case causality assessment.